Joseph

PXF – photo of lens

- What do you see?
- What else would you usually see in this syndrome?
- Patient profile?
- How does this differ from PDS in terms of signs and patient profile?
- What is your work up?
- Would you treat?
- What IOP would you consider treatment?
- If it was 10mmHg difference would you treat?
    - What drops
        - § PGA contraindications
    - What if PGA contraindicated
        - § Beta blockers contraindications
    - What drops would you choose in pregnancy?
        - § What class is bimonidine
- When do you review?
- If no glaucoma when do you review?

MK – marginal vs bacterial

- What is your diagnosis?
- How can you differentiate between microbial and marginal keratitis?
- How would you treat bacterial?
- When do you refer?
    - Why do you refer old people?
- What is the dual therapy? And how they work?
- What the difference between gram positive and gram negative?

Jason you’re a god mate
Anna just showed me disc photos and asked me to describe. Don't mention size coz you can't tell without fovea in shot (which is what i did lol) and then said c/d, there was a splinter haem inferiorly in one eye. Then she gave me age, iop, cct and fields were clear
the cct were 585 which i said was thicker end of normal and then she asked for range, i said 510-580 with a mean of 550. she was like thats a big range where are you getting that from. i said nhmrc but not sure if right or not

then chose a PGA she asked contraindications dug myself a hole with pregnancy coz i said only in last trimester its bad. she was like which ones can you use, instead PGAs. i was like no coz then you would have to change drugs during pregnancy. she asked what category i was like b3 i think
then she asked next choice i said beta blocker then she asked contraindications. i said asthma, COPD, heart problems, systemic beta blockers, diabetics due to masking hypoglycaema. then she asked what else
she also asked specifically what heat condition. but at this point she was like youre doing really well. Also i forgot what C stands for in COPD, i said coronary but its chronic

Lily just gives you a photo and asks what is the most likely diagnosis? Mine was Uveitis
and then what is your workup? this includes history questions and clinical shit like what you would look for like corneal signs. i mention NaFl to exclude bacterial keratitis. i said evert lids. I said do dfe to check for intermediate and posterior involvement and said i’d use atropine (to break synechiae, support BAB, reduce ciliary spasm etc). Then use phenylephrine if syneichiae not broken. She asked what is good about using atropine and phenylephrine work well together. Why not atropine and cyclo together. What would you do if atropine and phenyl didnt break the synechiae what would you do?
When would you refer? I said systemic cause, chronic, recurrent, herpes, not responding to treatment, hypopyon, posterior involvement. But she wanted something more...

Anna: Picture of optic nerve with L disc haemorrhage

1. Describe the disc (no fovea so cant say how big it is, NRR, PPA, disc haemorrhage, difficult to see NFL)
2. DDX: NTG, POAG, PVD, diabetes, HTN, physiological cupping (but haem)
3. Then revealed information: 26 IOP OU, CCT 585, FHx mum glaucoma, VF clear
4. Work up: IOP phasing, gonio, repeat VF for baseline, OCT, DFE (see if PVD but px asymptomatic so unlikely)
5. DX: OHT high risk of glaucoma due to FHx, high IOP, and if drance haem > risk for having glaucoma and progression of glaucoma. Could also be pre-perimetric glaucoma, overestimated IOP (Moorefields vs. NHMRC)
6. Management: make sure not PVD, GP for work up (HTN, DM), OCT in 3 months to see if RNFL loss. If a defect tx because tx reduces risk of developing glaucoma in 50% OHT
7. If you were to treat, what would you treat with?
    a. PGA: latanoprost 0.005% at night. contraindication? onset of effect, max effect, IOP reduction
    b. BB: timolol 0.25%. Why? Because most effective for light coloured eyes. Why in the morning? Because IOP is highest in the morning due to aq production. Not as effective at night because not as much aq production. I mentioned something about hypotension at night which i retracted… LOL contraindication, onset of effect
8. Steroid response- what percent of population responds, who is most likely to, when does it happen

Lily: Picture of eye with synechiae

1. Give a diagnosis. AAU
2. Work up: Cells and flare, fluorescein, gonio, IOP, DFE, 5 B's
3. MX: break synechiae. With atropine, if that doesnt work what next? Phenylephrine. Why does that work, because different mechanism of action. If that doesnt work, pledge- soak cotton with phenylephrine 10 and put in fornix for 15min?? moderate: predforte q1h until 2 step improvement in SUN, qid until zero and taper dropping a drop each week. Atropine 2X day. What are the contraindications for atropine: I said narrow angles, old people with cardiovascular problems
4. When would you refer: not improving or responding to tx, systemic assoc., posterior involvement, hypopyon, chronic, recurrent, and i should have said bilateral